EVERY STAGE HEALTH

From the American College of Obstetricians & Gynecologists

FAQs

Endometriosis

Overview

What is endometriosis?

Endometriosis is a chronic (long-lasting) condition that causes the type of tissue that forms the lining of the uterus (the endometrium) to grow outside the uterus. About 1 in 10 women of reproductive age develop endometriosis.

Everyone's experience of endometriosis is different. Sometimes it causes only mild discomfort. Or it can cause pain that is so severe that it prevents you from doing your normal activities. Endometriosis is also a common cause of infertility.

What are the symptoms of endometriosis?

The most common symptom of endometriosis is chronic pelvic pain. Other symptoms include:

- Dysmenorrhea—Pain during the menstrual period. If you have endometriosis, dysmenorrhea often becomes worse over time.
- Dyspareunia—Pain during sexual intercourse (vaginal sex)
- Pain during urination or bowel movements
- Other digestive system problems like constipation and bloating
- Infertility

All of these symptoms can affect your quality of life and make it hard to do the things you need or want to do.

I have a lot of pain. Does this mean I have severe endometriosis?

The amount of pain does not always match the severity of the condition. For example, you could have slight pain but a large number of lesions and many adhesions . You could have severe pain but a small number of lesions and few adhesions.

You may also have no symptoms. It's possible that you may find out that you have endometriosis only after you are unable to get pregnant or while having surgery for another reason.

Where is endometriosis often found?

In endometriosis, areas of endometrial tissue (often called lesions) are found outside the uterus, usually inside the pelvis and abdominal cavity. Endometriosis lesions most often appear in the following places:

- Peritoneum (inside the abdomen)
- Ovaries
- Fallopian tubes
- Outer surfaces of the uterus, bladder , ureters , intestines, and rectum
- Cul-de-sac (the space behind the uterus)

Lesions can grow on the outermost surface of organs or can grow deeper into the walls of some organs like the bladder or intestines. Lesions can be quite small or grow to the size of an orange or larger. In rare cases, endometriosis tissue may be found in parts of the body other than the pelvis, such as the diaphragm.

You may also develop endometriomas, a type of ovarian cyst that can cause pain and infertility.

What is the role of estrogen in endometriosis?

Endometriosis responds to changes in estrogen, which is a hormone made in the ovaries. The areas of endometriosis tissue may grow and bleed like the uterine lining does during the menstrual period. The tissue around the lesions can become irritated, inflamed, and swollen.

The breakdown and bleeding of this tissue each month can also cause scar tissue, called adhesions, to form. Sometimes adhesions can cause organs to stick together. The bleeding, inflammation, and scarring can cause pain, especially before and during menstruation.

Who is usually diagnosed with endometriosis?

Endometriosis is most often diagnosed during the reproductive years, but it has also been found in teens and postmenopausal women. Endometriosis may be partly inherited—passed down from parent to child through genes. If you have a close relative with endometriosis, you are more likely to have it too.

Are there other risk factors?

You are more at risk for developing endometriosis if you

- Started having a period very young (before you were 10 years old)
- Have a short menstrual cycle (less than 26 days from the beginning of one period to the beginning of the next)
- Have not had children
- Are underweight
- Were born with differences in the shape of your uterus or other reproductive organs

What is the connection between endometriosis and infertility?

Endometriosis is a common cause of infertility. Up to 6 in 10 women with infertility have endometriosis.

- In severe endometriosis, the fallopian tubes may be blocked by adhesions or scar tissue, which may prevent an egg from moving to the uterus.
- In less severe cases, inflammation may damage the sperm or egg. Inflammation may also interfere with the sperm or egg's movement through the fallopian tubes and uterus.

Does endometriosis get better after menopause?

Endometriosis symptoms usually go away or get better after menopause . After menopause, the ovaries stop making estrogen. Without estrogen, endometriosis growth generally stops and the lesions usually get smaller.

Diagnosis

What is the first step to getting diagnosed with endometriosis?

If you have pain and other symptoms of endometriosis, your obstetrician–gynecologist (ob-gyn) first may ask for a detailed health history and a list of your symptoms. They may also do a physical exam, including a pelvic exam .

Many of the symptoms of endometriosis are similar to those of other problems, such as irritable bowel syndrome, urinary tract problems, and infections. Your ob-gyn will need to rule out these other causes.

What kinds of imaging tests can help find endometriosis?

The best test for finding endometriosis is a transvaginal ultrasound exam . You can have this test even if the pelvic exam was normal, because endometriosis lesions can be small or in places that a pelvic exam can't reach.

If you are a teenager or have sexual trauma, you may be able to have a different kind of ultrasound exam instead. The ultrasound may be on your abdomen or your perineum .

Sometimes a magnetic resonance imaging (MRI) test may also be needed to find endometriosis that is deeper in the pelvis or outside of the pelvis. You may also need other imaging tests like cystoscopy or colonoscopy to check inside the bladder and colon.

You can start getting treatment for your symptoms right away, even before having an imaging test. This can help you start feeling better while you wait for the results to come in.

Even if an imaging test does not show endometriosis, you may still have it. Some kinds of lesions are difficult to see on imaging.

I've heard that I need surgery before I can get a diagnosis. Is this true?

It is true that the only way to confirm that you have endometriosis is with surgery, also called a "diagnostic laparoscopy ."

But you are not required to have this surgery. If you like, you can start medical treatment for many endometriosis symptoms without having this test.

Your ob-gyn may recommend a diagnostic laparoscopy if

- they think you might have another condition instead of or in addition to endometriosis
- you want to have a definitive diagnosis before you start treatment
- you want to have a diagnosis and lesion removal at the same time

- your medical treatment isn't working

What happens during a laparoscopy?

Laparoscopy involves making a small incision (cut) in your abdomen, often near your belly button. A device called a laparoscope is inserted through this incision into your abdomen. The laparoscope allows the surgeon to view your pelvic organs.

You are usually given general anesthesia for this procedure.

Sometimes a small amount of tissue is removed. This is called a biopsy .

If your surgeon finds endometriosis lesions or endometriomas during the laparoscopy, they can try to remove them to try to avoid needing another surgery soon after. They should talk with you about this option first so that you know what to expect.

You may be able to choose a surgeon who specializes in endometriosis. This kind of surgeon can plan to diagnose and treat your endometriosis at the same time.

Treatment

What kinds of treatments are available for endometriosis?

Endometriosis may be treated with medication, surgery, or both. Endometriosis does not have one easy treatment. You may need several kinds of treatment, some of them at the same time.

The kind of treatment you have depends on several factors, including

- how severe your endometriosis is
- what symptoms you have
- whether or not you want to get pregnant in the future
- your treatment preferences

How do hormonal medications help with endometriosis?

Hormones may help slow the growth of the endometriosis lesions and may keep new lesions from forming. This can help manage pain symptoms, but these medications typically do not get rid of endometriosis tissue that is already there.

As with most medications, many of these options can cause side effects. These medications do not relieve pain for everyone. And the pain often returns after the medications are stopped.

Can NSAIDs help with endometriosis pain?

If you have endometriosis pain but are trying to get pregnant, your ob-gyn may recommend that you try nonsteroidal anti-inflammatory drugs (NSAIDs) first. NSAIDs may relieve pain, but they do not treat any other symptoms of endometriosis.

How can combined hormonal birth control methods help endometriosis?

Combined hormonal birth control methods contain the hormones estrogen and progestin . They come in the form of pills, a skin patch, or a vaginal ring. They can help

- control the menstrual cycle
- reduce inflammation in areas of endometriosis
- keep periods regular, which can relieve pain

You can also use these methods to reduce the number of periods that you have or stop them altogether.

How can progestin-only birth control methods help endometriosis?

Progestin-only birth control methods can be used to manage endometriosis lesions. They come in the form of pills, a shot, an implant, or the hormonal intrauterine device (IUD) .

You can also use these methods to reduce the number of periods that you have or stop them altogether.

How can GnRH agonists help endometriosis?

Gonadotropin-releasing hormone (GnRH) agonists decrease estrogen levels by stopping the function of the ovaries. This causes a short-term condition that is much like menopause. You will not have periods and will not be able to get pregnant while you are taking GnRH agonists. In most cases, GnRH agonists cause endometriosis lesions to shrink, which can relieve pain.

GnRH agonists can be given as a shot, implant, or nasal spray.

Side effects of this medication may include:

- Hot flashes
- Headaches
- Vaginal dryness
- Decrease in bone density

You will likely also have “add-back” therapy while you are on GnRH agonists. This therapy uses small doses of estrogen or other hormones to reduce the side effects of GnRH agonists.

Treatment with GnRH agonists usually lasts 3 to 6 months. In some cases, you may need longer treatment. After stopping GnRH agonists, your periods usually come back within 6 to 10 weeks. If you need treatment for longer than 6 months, add-back therapy will be needed for a while after you stop taking GnRH agonists.

What about surgical treatments?

Surgery can be done to relieve pain and improve fertility. During surgery, endometriosis lesions can be removed by cutting or with a laser.

Will surgery cure me?

Most of the time, surgery brings relief from endometriosis pain. But up to 8 in 10 women have pain again within 2 years of surgery. This may be because of endometriosis that was not visible or could not be removed at the time of surgery. It may also be because endometriosis is a long-term condition and new lesions can form.

The more severe your endometriosis is, the more likely it is to return. Taking birth control pills or other hormonal medications after having surgery may help keep you pain-free for longer.

What should I know about hysterectomy?

If your pain is severe and does not go away after treatment, a hysterectomy may be a treatment option. Endometriosis is less likely to lead to future pain if your ovaries are removed at the time of hysterectomy.

After a hysterectomy, you will no longer have periods or be able to get pregnant. If your ovaries are also removed and you have not yet gone through natural menopause, you will experience early menopause symptoms caused by lack of estrogen. You may need to take hormone therapy to manage these symptoms.

How can I cope with my symptoms?

Endometriosis is a long-term condition that does not have one easy cure. Many women have symptoms off and on until menopause. But you can find treatments that will help manage your symptoms.

You may need several kinds of treatment, some of them at the same time. You can work with your ob-gyn to find treatment options that are right for you.

You may be able to find support groups in your area or online. These groups may have tips about exercise or relaxation techniques that can help you cope with pain and discomfort. With these strategies and an effective treatment plan, you can find relief.

Glossary

Adhesions: Scars that can make tissue surfaces stick together.

Biopsy: A minor surgical procedure to remove a small piece of tissue. This tissue is examined under a microscope in a laboratory.

Bladder: A hollow, muscular organ that holds urine.

Chronic Pelvic Pain: Pain in the pelvic region that lasts for more than 6 months.

Colon: The large intestine.

Colonoscopy: An exam of the large intestine using a small, lighted instrument.

Cyst: A sac or pouch filled with fluid.

Cystoscopy [sis-TAH-skuh-pee]**:** A procedure that looks at the inside of the urethra and bladder.

Digestive System: A system made up of the stomach, bowels, liver, gallbladder, and pancreas. This system breaks down food and removes waste from the body.

Dysmenorrhea [dis-men-uh-REE-uh]**:** Discomfort and pain during the menstrual period.

Dyspareunia [dis-par-OOH-nee-uh]**:** Pain with vaginal sex.

Egg: A reproductive cell made in and released from the ovaries. Also called the ovum.

Endometriosis [en-doh-mee-tree-OH-suhs]**:** A condition that causes tissue that lines the uterus to grow outside of the uterus, usually on the ovaries, fallopian tubes, and other parts of the pelvis.

Endometrium [en-doh-MEE-tree-uhm]**:** The lining of the uterus.

Estrogen: A sex hormone made in the ovaries.

Fallopian Tubes [fuh-LOH-pee-uhn]**:** Tubes through which an egg travels from the ovary to the uterus.

General Anesthesia: The use of medications that create a sleep-like state to prevent pain during surgery.

Genes: Segments of DNA that contain instructions for the development of a person's physical traits and control of the processes in the body. The gene is the basic unit of heredity and can be passed from parent to child.

Gonadotropin-Releasing Hormone (GnRH) Agonists [goh-nad-uh-TROH-puhn ri-LEE-sing]**:** Medical therapy used to block the effects of certain hormones.

Hormone: A substance made in the body that controls the function of cells or organs.

Hormone Therapy: Treatment with hormones, such as estrogen and progestin, to help treat the symptoms of menopause.

Hysterectomy [his-tuhr-REK-tuh-mee]**:** Surgery to remove the uterus.

Infertility: The inability to get pregnant after 1 year of having regular vaginal sex without the use of birth control, or after less than 1 year based on factors like medical history, age, or test results.

Inflammation: Pain, swelling, redness, and irritation of tissues in the body.

Intrauterine Device [in-truh-YOO-tuhr-ruhn] **(IUD):** A small device that is inserted and left inside the uterus to prevent pregnancy, lighten or stop periods, or reduce the risk of endometrial cancer.

Laparoscope [LAP-uh-ruh-skohp]**:** A thin, lighted telescope that is inserted through a small incision (cut) in the abdomen to view internal organs or perform surgery.

Laparoscopy [lap-uh-RAH-skuh-pee]**:** A surgical procedure using a thin, lighted telescope called a laparoscope. The laparoscope is inserted through a small incision (cut) in the abdomen and used to view the pelvic organs. Other long, thin instruments can be used with it to perform surgery.

Magnetic Resonance Imaging (MRI): A test to view internal organs and structures by using a strong magnetic field and sound waves.

Menopause: The last menstrual period, often happening around age 51. Menopause can only be confirmed after 1 year of no periods.

Menstrual Cycle: The monthly changes that prepare the body for a possible pregnancy. A menstrual cycle is defined as the first day of menstrual bleeding of one cycle to the first day of menstrual bleeding of the next cycle.

Menstrual Period: The monthly shedding of blood and tissue from the uterus. Also called menstruation.

Nonsteroidal Anti-Inflammatory Drugs (NSAIDs): Medications that relieve pain by reducing inflammation. Many types are available over the counter, including ibuprofen and naproxen.

Obstetrician–Gynecologist (Ob-Gyn): A doctor with medical and surgical training and education in the female reproductive system.

Ovaries: Organs that contain the eggs necessary to get pregnant. Ovaries also make important hormones, such as estrogen, progesterone, and testosterone.

Pelvic Exam: A physical examination of the pelvic organs, including the vagina, cervix, uterus, and ovaries.

Perineum [pair-uh-NEE-uhm]**:** The area between the vagina and the anus.

Peritoneum [pair-uh-tuh-NEE-uhm]**:** The membrane that lines the abdominal cavity and surrounds the internal organs.

Postmenopausal: Being in postmenopause, the stage of life that comes after menopause.

Progestin [proh-JES-tuhn]**:** A synthetic form of progesterone that is similar to the hormone made naturally by the body.

Rectum: The last part of the digestive tract.

Sexual Intercourse: The act of the penis entering the vagina. Also called vaginal sex.

Sperm: A cell made in the testicles that can fertilize an egg.

Transvaginal Ultrasound Exam: A type of ultrasound where the device is placed in the vagina.

Ultrasound Exam: A test that uses sound waves to examine inner parts of the body. During pregnancy, ultrasound can be used to check the fetus. Also called ultrasonography or sonography.

Ureters [YUR-uh-tuhrz]**:** A pair of tubes, each leading from one of the kidneys to the bladder.

Uterus: A muscular organ in the female pelvis. During pregnancy, this organ holds and nourishes the fetus. Also called the womb.

If you have further questions, contact your ob-gyn.

Don't have an ob-gyn? Learn how to find a doctor near you.

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